

placebo or standard-care-only comparator, were not eligible, because such trials address the choice between TCM regimens rather than the effect of adding TCM to standard care.

Outcomes

Functional status, neurological deficit, activities of daily living, motor recovery, recurrent vascular events, mortality and adverse events. Trials whose only outcomes were biomarkers or imaging measures were eligible but are reported separately, because such outcomes are indirect for the clinical questions of interest.

Study design

Randomized controlled trials. Non-randomized studies, single-arm studies, protocol publications without results, and observational extension studies of randomized trials were not eligible. Where a trial generated more than one publication, the report of its primary outcome was the primary reference and secondary analyses were used only for supporting detail.

All clinical phases were eligible, a deliberate decision taken to characterize the breadth of the randomized evidence. Because acute treatment, early rehabilitation and secondary prevention are different clinical questions, the synthesis is stratified by clinical phase throughout and no result is combined across phases. Where a trial included more than two arms, only the comparison of the TCM formulation against placebo or standard care contributes to the synthesis.

Information sources and search

We searched seven bibliographic databases (PubMed, Embase, Web of Science, the Cochrane Central Register of Controlled Trials, China National Knowledge Infrastructure, Wanfang Data and SinoMed) and one clinical trial registry (the Chinese Clinical Trial Registry) to 8 April 2026. MEDLINE content was searched through the PubMed interface and is not counted as a separate database. The reference lists of relevant reviews and of included reports were screened. No language restriction was applied.

The original database-specific search histories and exact executed strings were not retained in the files available for this revision. Accordingly, the search cannot be reproduced exactly from the present record. The databases, registry, final search date and broad concepts are reported, but no retrospectively reconstructed syntax is presented as if it were the search originally run. This limitation is considered when interpreting the completeness of the review. Further information on the searched sources and the status of the unavailable execution details is provided in [Supplementary Appendix 1](#).

Zotero and Rayyan were used for reference management and screening, respectively. Exact software versions were not retained and are therefore not reported.

Selection and data collection

Records from all sources were imported into reference-management software and deduplicated. Titles and abstracts were screened against the eligibility criteria, and records passing that stage were retrieved in full text and assessed against the same criteria. Data were extracted into a structured form covering study design and setting, participant eligibility and baseline characteristics, intervention composition, dose, route, frequency and duration, comparator and background care, follow-up, primary and secondary outcomes, analysis populations, effect estimates, adverse events, funding and trial registration. Deduplication, title and abstract screening, full-text screening and data extraction are reported as four separate processes.

The surviving review records did not permit independent confirmation of whether title-and-abstract screening, full-text screening and data extraction were each completed in duplicate. The manuscript therefore does not claim duplicate review for these stages. This loss of process documentation reduces auditability and is acknowledged as a limitation. The data fields and figure-level evidence dataset are provided in [Supplementary Data File 1](#).

Risk of bias

Risk of bias was assessed with RoB 2 (21) for one explicitly specified result in each trial, namely the effect of assignment to the intervention on that trial's primary outcome. Where a trial did not designate a primary outcome, the most clinically relevant reported outcome was selected and identified as the specified result. Assessments used the official signaling questions and algorithm across the five domains, and the overall judgment follows the algorithm, so that it is never more favorable than the least favorable contributing domain. Domain judgments, rationales and supporting source text are in [Supplementary Table 2](#), and [Figure 1](#) and [Table 1](#) are generated from that single dataset.

Missing information was handled within the domain to which it belongs. Absence of a reported detail was not treated as automatic evidence of high risk, and neither was it treated as reassurance; where a signaling question could not be answered from the report, protocol or registry entry, the domain was judged to raise some concerns.

Risk-of-bias judgments were reassessed during revision from the published reports, available protocols and registry records and were checked against the RoB 2 algorithm. The available documentation did not establish an independent duplicate assessment; the judgments should therefore be interpreted as a transparent reassessment rather than a documented consensus of two independent assessors.

Synthesis and certainty of evidence

We did not perform a meta-analysis and report no pooled summary estimate. The decision rests on clinical and methodological differences between the trials, not on data availability. The nine included trials evaluate eight formulations